

Cutaneous Adverse Reactions Associated with Novel Antihypertensives: A Clinical Series of Three Cases

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ABSTRACT

We document three distinct individual cases of severe cutaneous drug reactions occurring secondary to novel antihypertensive therapies across two university medical centers. Case 1 describes a 45-year-old male with severe Erythema Multiforme Major. Case 2 describes a 62-year-old female presenting with subacute cutaneous lupus erythematosus. Case 3 documents a 38-year-old female with acute urticaria and periorbital angioedema. All cases responded favorably to prompt drug cessation.

INTRODUCTION

The expansion of novel antihypertensive pharmacotherapies has coincided with sporadic reports of delayed-type hypersensitivity reactions. We present three well-characterized independent patient cases identified during a 6-month surveillance period.

PATIENT CASE 1

A 45-year-old male (Patient A.J., weight 81 kg) with stage II hypertension was prescribed Cardioril 20 mg once daily. On day 22 of therapy, he presented with extensive symmetrical target lesions with central blistering on the dorsal hands, forearms, and lower extremities, accompanied by severe oral mucosal erosions. Diagnosis: Erythema Multiforme Major. Histopathology confirmed apoptotic keratinocytes with vacuolar basal interface dermatitis. Cardioril was discontinued, and systemic methylprednisolone was initiated. Complete resolution was achieved within 16 days.

PATIENT CASE 2

A 62-year-old female (Patient B.L., weight 64 kg) commenced Corzapan 10 mg daily for essential hypertension. After 8 weeks of therapy, she developed widespread annular, polycyclic scaly erythematous plaques distributed across sun-exposed areas of the upper back and shoulders. Serology revealed high-titer anti-Ro/SSA antibodies (>240 U/mL) and ANA 1:320. Diagnosis: Drug-induced subacute cutaneous lupus erythematosus (SCLE). Corzapan was discontinued. Lesions resolved over 8 weeks with topical tacrolimus.

PATIENT CASE 3

A 38-year-old female (Patient C.M., weight 59 kg) with mild hypertension was initiated on Cardioril 10 mg daily. Approximately 90 minutes following her initial dose, she developed acute generalized urticaria with severe pruritus, dysphonia, and marked bilateral periorbital angioedema. Emergency management with intramuscular epinephrine 0.3 mg and intravenous diphenhydramine 50 mg aborted progression to respiratory compromise. Cardioril was permanently avoided.

PHARMACOVIGILANCE DISCUSSION

These three cases underscore the clinical necessity for distinct Individual Case Safety Report (ICSR) submission for each patient, despite publication within a unified clinical series. Each case features unique demographic profiles, differing onset latencies (ranging from 90 minutes to 8 weeks), distinct MedDRA clinical reaction phenotypes, and independent de-challenge timelines.

Safety reporting departments must ensure that multicase journal articles are disaggregated into separate regulatory cases to prevent signal dilution.

REFERENCES

1. Roujeau JC. Immune mechanisms in drug eruptions. Clin Dermatol. 2020;38(6):679-688.
2. EMA Guideline on Good Pharmacovigilance Practices (GVP) Module VI — Collection, verification and presentation of adverse reactions.